

SMART MOSQUITO CONTROL SYSTEM

25 – 26J – 369

Project Proposal Report

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Specialized in Information Technology

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Sri Lanka

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**SMART MOSQUITO CONTROL SYSTEM –
MOBILE/WEB HEALTH DATA INTEGRATION
PLATFORM**

25 – 26J – 369

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Declaration

We declare that this is our own work, and this proposal does not incorporate without acknowledgement any material previously submitted for a degree or diploma in any other university or Institute of higher learning and to the best of our knowledge and belief it does not contain any material previously published or written by another person except where the acknowledgement is made in the text.

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The supervisor/s should certify the proposal report with the following declaration.

The above candidates are carrying out research for the undergraduate Dissertation under my supervision.

Signature of the supervisor:

Date:

Signature of the co-supervisor:

Date:

Abstract

The project will develop and deploy a web and mobile-web based health data integration system to serve Sri Lanka's mission of preventing mosquito-borne diseases such as dengue and chikungunya. The system will be developed for harvesting real-time case reports from hospitals, aggregating them in a centralized cloud database, and visualizing them via an interactive outbreak heatmap. Through the process of converting raw hospital data into clear visualizable insights, the platform will be able to easily advise the public and the health officials on which areas are at risk. One of the unique aspects of this platform is that it will make use of forecasting based on machine learning, where historical hospital data coupled with environmental data such as rainfall and temperature will be applied to forecast future patterns of outbreaks. This predictive functionality allows the system not just to report where outbreaks exist at any moment, but also to warn users about where outbreak areas will be high-risk in the near future. For maximum efficiency, the site will have a notification and alert system that provides real-time outbreak alerts as well as predictive alerts. These notifications will be sent to health administrators to enable prompt decision-making as well as to the public at large to trigger preventive action. On top of that, the website will include a multilingual awareness module, offering real-life prevention tips in Sinhala, Tamil, and English, designed for Sri Lankan communities. Even a simple chatbot in the mobile app will render the system interactive where users can pose questions such as "Is there dengue in my area?" or "What are the symptoms?", and get clean, localized answers.

With its integration of real-time hospital information, predictive analytics, interactive visualization, and simple awareness tools, this project presents Sri Lanka with a real-world, localized, and cutting-edge solution for reacting to mosquito-borne disease outbreaks.

Keywords: Heat mapping, Localization undefined

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1. Introduction

Sri Lanka is still plagued by repeated epidemics of mosquito-borne disease, primarily dengue, which is a major public health priority. Seasonal epidemics exact heavy burdens on hospitals and populations every year, mostly during monsoons when breeding sites for mosquitoes are more common. The World Health Organization (WHO) includes dengue as one of the fastest-spreading mosquito-borne diseases in the world, and Sri Lanka is one of the highest-risk countries for repeated epidemics [1]. Manual processes such as fogging, campaigning, and hand reporting facilitate the burden to some degree but are mostly of a reactive kind and prone to reporting delay. A crucial shortfall is that hospital real-time location information cannot be conveyed to health authorities or the public in an understandable manner.

As a response to this challenge, in this project, an integration platform for web and mobile health data is proposed that transforms raw case hospital data into timely, actionable, and predictive outbreak data. Six key elements are the foundation of the system:

1. Hospital Data Integration Module – Retrieves real-time dengue and chikungunya case data from hospitals and consolidates them within a central cloud database.
2. Heatmap Module – Displays hospital information on an interactive, dynamic heatmap so that users can easily get a glimpse of low, medium, and high-risk regions.
3. ML-Based Predictive Analytics Module – Utilizes historical case trends in hospitals and weather conditions such as rainfall and temperature to forecast possible outbreaks well ahead of time.
4. Alert & Notification System – Provides real-time notification to the public health officials and the general public, such as weather warning when the region is high-risk.
5. Prevention & Awareness Module – Provides local health tips in Sinhala, Tamil, and English, adapting to Sri Lanka's environmental and cultural conditions.
6. Chatbot Module – Interactive in-app personal assistant which allows a user to query, for instance, "Is dengue in my area?" or "What are the symptoms?" and receive concise, focused answers.

All of these combined constitute an integrated system which not only captures ongoing outbreak activity but also predicts future risk, provides real-time know-how, and instigates

preventive behavior. In contrast with existing solutions most likely to be static, late, or geographically out of context, this web site focuses on speed, forecastability, multilinguality, and user participation. By bringing together hospital data and cutting-edge technologies such as machine learning, map-interactive awareness, and chat-awareness, this project is an applied and innovative step ahead for Sri Lanka in the context of insect-borne diseases.

1.1 Background & Literature survey

Dengue and chikungunya, mosquito-borne diseases, remain a major public health problem in Sri Lanka. Seasonal epidemics are concurrent with rainfall and poor management of water, and urban centers have the highest numbers of reported cases. The Epidemiology Unit at the Ministry of Health reported more than 70,000 suspect dengue cases in 2023, a figure that reflects both the scale and spread of the problem [2]. When there is a sudden increase in cases, the hospitals are stretched to the limit, and the communities are seriously disrupted. While routine measures like fogging of mosquitoes, inspections, and awareness campaigns are consistently conducted, they tend to be reactive rather than proactive. The main challenge is the time lag between hospital reporting of new cases and the availability of this information to the public in a readily accessible format that people can grasp and act upon.

They have attempted to bridge this gap with computer-based programs. In Singapore, the National Environment Agency publishes an internet-based map of dengue cases that recognizes high-risk clusters and posts new cases [3]. Spain's Mosquito Alert project has tackled the problem differently by engaging the public in reporting the presence of mosquitoes, which is verified by experts and fed into a master database [4]. Both exercises illustrate the power of technology to raise awareness and pinpoint extreme limitations. The Singapore approach, while official, does not attempt to predict future epidemics. The Spanish approach is highly participatory but relies on citizen-based data, which tends to vary in reliability. Neither has been designed for a multilingual, resource-poor setting such as Sri Lanka.

In studies, there has been significant progress as well in using machine learning for predicting outbreaks of dengue. Chinese and Southeast Asian research has employed historical hospital case records along with weather data to come up with models that are able to forecast case numbers weeks in advance [5][6]. Decision trees, regression analysis, and time-series models such as LSTM have shown promising results in predicting outbreak trends. These approaches demonstrate the potential to move beyond static case reporting to predictive frameworks that can guide early intervention. However, most of these works remain in the world of academe and have not yet reached the status of being applied as tools ready for use by health officers or the general public.

The platform proposed here seeks to fill that chasm by combining hospital-reported data with predictive analytics and a communications system designed for Sri Lanka. It analyzes hospital data in real time, presents it as an interactive heatmap, and predicts with machine learning techniques possible outbreaks. It delivers them both to health officers, who need timely data to be able to control resources, and to the public, who benefit from clear and useful advice. To promote greater accessibility, the platform provides locally adapted counsel in Sinhala, Tamil, and English, and features a chatbot that allows users to ask direct questions. By doing that, the system leverages available global models but adapts them to local needs, so that outbreak information is not just available, but also understandable and actionable.

1.2 Research Gap

While Sri Lanka has a high incidence of dengue and chikungunya cases every year, there are few mechanisms in place to share outbreak information with the public and health officials. Hospital data is gathered, but not necessarily translated into a format that is available or actionable. Current methods of communication, such as official press statements or non-interactive announcements on government websites, are often outdated and slow. As a result, communities only hear about outbreaks once case numbers are already high, and preventive measures are less effective.

International experiences demonstrate the potential of technology here, although they also point out some weaknesses. Applications such as Singapore's dengue cluster map provide official report news but are reactive with no built-in predictive smarts. People science projects such as Spain's Mosquito Alert crowdsourced reporting, but the reliability of that information is user-driven and not consistently high-quality. Significantly, neither of these approaches has been formulated to meet Sri Lanka's specific needs, where language diversity, varying degrees of digital illiteracy, and rural internet penetration gaps present unique challenges.

Research has shown that outbreaks of dengue could be forecasted with reasonable accuracy with the help of machine learning models by combining historical case data and environmental parameters. The model predictions are never utilized as a part of an operational platform that could be accessed by health officers or the public at large. There is, thus, a disconnect between research output and real-world applicability.

Therefore, the gap in research is the absence of a real-time, predictive, and user-friendly health data integration platform for Sri Lanka that:

- Effectively gathers and processes hospital-reported case data.
- Visualizes outbreak trends with dynamic heatmaps.
- Applies machine learning to forecast future risk.

- Transmits findings through timely alerts and multi-language awareness materials.
- Provides interactive support, e.g., a chatbot, to engage with users.

Closing this gap will not only increase the country's counter to mosquito-borne disease but also demonstrate how predictive analytics can be incorporated into public health messaging systems within low-resource settings.

Table 1 - High-Level Representation of the Research Gap

Feature	WHO Surveillance Reports	Singapore NEA Dengue Maps	Mosquito Alert (Citizen Science)	AI-Based Prediction Models	Proposed Health Data Integration Platform
Real-time hospital case integration	✗	✗	✗	✗	✓
Interactive outbreak heatmaps	✗	✓	✗	✗	✓
Predictive outbreak forecasting (ML)	✗	✗	✗	✓	✓
Multilingual awareness content	✗	✗	✗	✗	✓
Personalized user interaction (chatbot)	✗	✗	✗	✗	✓
Localized for Sri Lanka context	✗	✗	✗	✗	✓

From the comparison, it is evident that current systems either present aggregated data with a delay, limited geographical insights, or rely on crowdsourced reports without verification. They do not integrate real-time hospital data, predictive machine learning, multilingual consciousness, and interactive user interaction on one platform. The Health

Data Integration Platform proposed here is novel in the sense that it addresses all these gaps simultaneously and is technologically advanced as well as appropriately contextual to Sri Lanka, which needs localized and real-time dengue warnings.

1.3 .Research Problem

Sri Lanka continues to face recurring outbreaks of mosquito-borne diseases such as dengue and chikungunya, yet existing systems fail to provide timely and predictive information. This creates several critical questions, along with the ways this project aims to address them:

1. **How can hospital-reported case data be made available in real time?**
At present, data reaches the public and health officers with delays through media or official updates. This project addresses the issue by introducing a **Hospital Data Integration Module** that collects and stores data in a centralized cloud system for immediate use.
2. **How can outbreak information be presented so that communities and officials can easily interpret it?**
Current reports are often text-based or broad regional summaries. The proposed system introduces an **Outbreak Heatmap Module** that visually displays hotspots, allowing users to quickly identify high, medium, and low-risk zones.
3. **How can upcoming outbreaks be predicted rather than only tracked after they occur?**
Existing tools are largely reactive. This project includes an **ML-Based Predictive Analytics Module**, which analyzes historical hospital data and environmental factors such as rainfall and temperature to forecast future outbreak risks.
4. **How can users and officials be notified quickly enough to take action?**
Without timely alerts, communities often miss the opportunity for early prevention. A **Notification and Alert System** in this project ensures both health officers and the public receive real-time and predictive warnings tailored to their location.
5. **How can awareness campaigns be more effective and accessible?**
Current communication often lacks multilingual support and is not interactive. The project incorporates a **Prevention & Awareness Module**, which provides localized tips in Sinhala, Tamil, and English, and a **Chatbot Module**, which allows users to ask questions and receive clear, practical answers.

Through these solutions, the research directly responds to the shortcomings of existing systems by creating a comprehensive, real-time, and predictive outbreak management platform designed for Sri Lanka.

2. Objectives

2.1 Main Objectives

The main objective of this research is to design and create an internet and mobile-enabled health data aggregation platform that reports dengue and chikungunya hospital-reported cases in real-time, secures it on a central, secure cloud database, and provides outbreak data as an interactive, GIS-based heatmap for visualizing high-risk, medium-risk, and low-risk areas in an easily readable format. The system will also have a machine learning component that sweeps through historical hospital data with climatic variables such as rain, temperature, and humidity to forecast better probabilities of likely future outbreak risk. In addition to prediction and visualization, the system will have a notification and warning mechanism to issue early warning to health planners for planning and to warn the public of risk in their immediate locality. To motivate preventive behavior, the website will display culturally relevant, multilingual warning messages in English, Tamil, and Sinhala languages and an interactive chatbot agent with which the users can interact to receive facts regarding outbreaks, symptoms, and preventive measures. By predictive analytics in real-time integration, visualization, and interactive communication, it aims to bridge the practice action-hospital-reported data gap and achieve timely disease control, while equipping communities to combat receding mosquito-borne disease transmission in Sri Lanka.

2.2 Specific Objectives

The objective that I need to accomplish is to develop a mobile and web-based health data integration platform that collects hospital-reported dengue and chikungunya case data, visualizes the outbreaks on heatmaps, and provides predictive insights for early prevention. In order to achieve this objective, I have further divided this component into six specific objectives, which include:

1. **Hospital Data Integration Module**

In order to create and implement a secure cloud-based solution to collect real-time dengue and chikungunya case reports from hospitals, validate inputs, and maintain an updated central database for tracking outbreaks.

2. **Interactive Outbreak Heatmap**

To create a GIS-capable heatmap that displays dynamic visualizations of hospital case information, depicting areas in terms of low, medium, and high-risk regions, and allowing users to contrast outbreak severity across different periods of time.

3. **Machine Learning Predictive Analytics**

To develop a machine learning-based predictive model (e.g., regression analysis, time-series forecast, or LSTM) that incorporates historic hospital data alongside environmental factors such as rain, temperature, and humidity to accurately predict outbreak patterns.

4. **Notification and Alert System**

To develop an automated real-time alert system offering outbreak alerts and predictive alerts to resource re-allocating health officers and to masses for preventive alerts through web and mobile.

5. **Multilingual Prevention and Awareness Module**

In order to create a content delivery system that offers localized prevention advice and outbreak notifications in Sinhala, Tamil, and English to cover and make it culturally responsive to Sri Lanka's populations diversity.

6. **Chatbot Assistant**

To integrate a chatbot as an AI technology in the mobile app that is capable

of answering user queries such as "Is there dengue near me?", "What are symptoms?" and "How do I prevent it?" thereby boosting engagement and visibility to health content.

3. Methodology

3.1 Overall System

The Smart Mosquito Control System integrates four bespoke modules from individual team members into a single unified ecosystem that addresses mosquito-borne disease control on all fronts:

1. Mosquito Identification Model

- Uses convolutional neural networks and audio classification to identify species of mosquitoes in real time.
- Provides labeled data (species type, quantity) to the system.
- Ensures disease-transmitting mosquitoes (*Aedes aegypti*, *Aedes albopictus*) are prioritized for monitoring.

2. Smart Mosquito Trap

- Creates an eco-friendly trap using UV light and CO₂ lures to preferentially target mosquitoes.
- Reduces chemical spraying reliance.
- Sensor traps forward capture data to the IoT network.

3. IoT Sensor Network & Dashboard

- Uses distributed sensors to measure mosquito density and activity.
- Forwards data to a cloud-hosted dashboard, aggregates data, and passes it to other parts.
- Provides environmental indicators to support ML outbreak forecasting.

4. Health Data Integration Platform

- Collects real-time hospital-reported dengue and chikungunya cases via APIs.
- Projects the outbreak data on heatmaps with risk categorization.
- Comprises ML-driven outbreak forecasting, notifications and alertness, multilingual awareness, and a chatbot for user interaction.

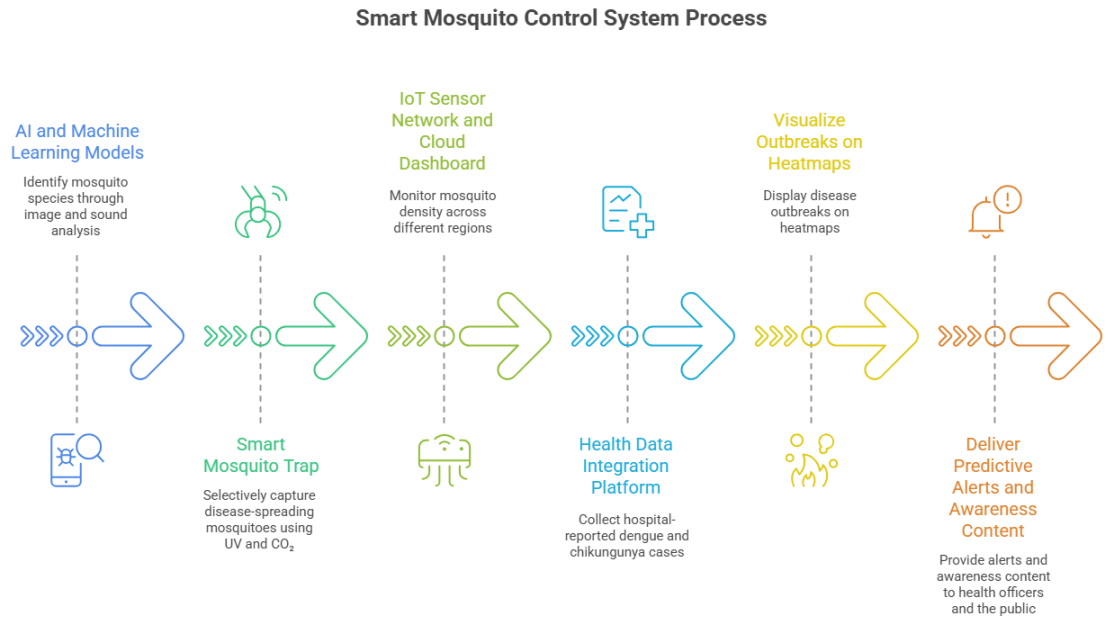


Figure 1 - SMART MOSQUITO CONTROL SYSTEM Diagram

3.2 Individual System Diagram

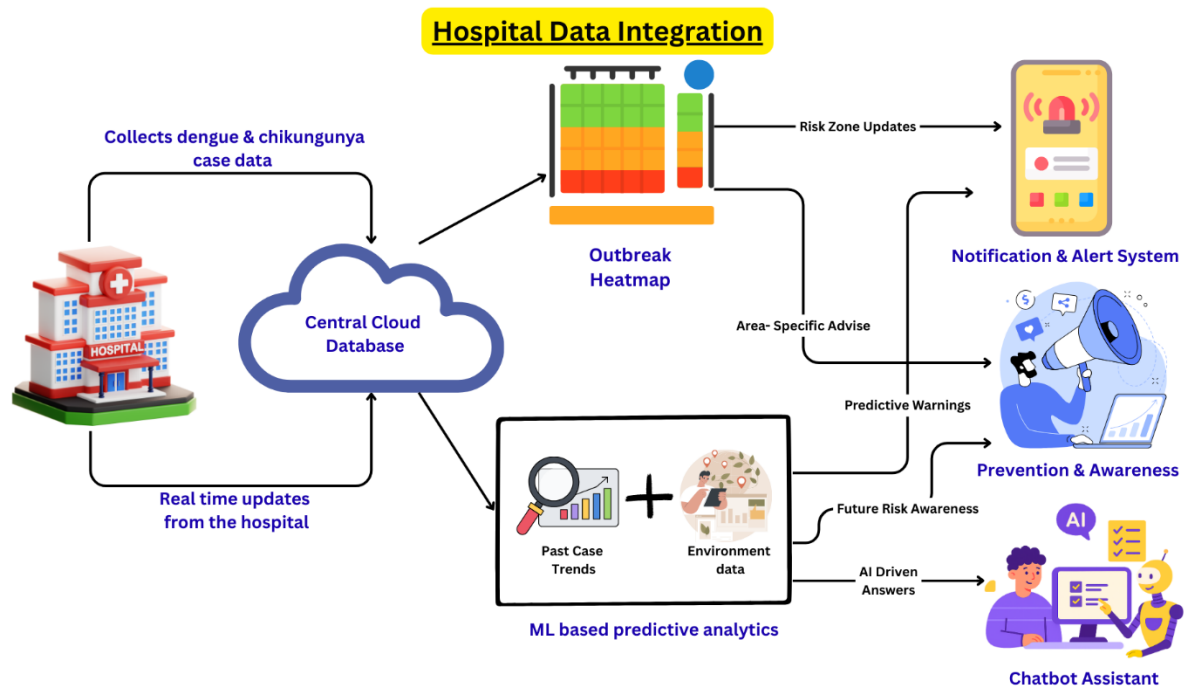


Figure 2 - Health Data Integration Platform system diagram

This diagram shows the platform for health data integration. Hospital case data are first accessed via APIs and stored in a centralized database. The data are processed and presented on an interactive outbreak heatmap, where risk levels for various areas are indicated in color. Case history data and environmental parameters are processed by an AI-based prediction module to predict likely zones of outbreak hotspots, also overlaying them on the heatmap. Thereafter, the system provides real-time messages and warnings to the general public as well as to the health authorities. An additional awareness feature is provided by a multilingual module providing preventive recommendations in English, Tamil, and Sinhala, and also a chatbot with functionality to ask for outbreak status, symptoms, and prevention. Therefore, technical end-users like health officers and citizens get timely, understandable, and actionable health information.

Key Steps in this Component:

1. Hospital Data Collection & Integration

- Secure APIs from hospitals for collection of dengue/chikungunya case reports.
- Store data in a central cloud database (MongoDB/PostgreSQL).
- Implement daily/real-time synchronization.

2. Data Processing & Heatmap Visualization

- Transform hospital case data to GIS-compatible formats.
- Use libraries like Leaflet.js or Mapbox to build an interactive outbreak heatmap.
- Highlight risk areas (low, medium, high) based on case density.

3. Machine Learning Prediction Module

- Train models on historical hospital case data + weather conditions (rainfall, temperature, humidity).

- Use regression, decision trees, or LSTM models to predict for outbreak hotspots.
- Overlay the predictions onto the heatmap to forecast high-risk areas.

4. Notification & Alert System

- Implement a push notification service (Firebase, OneSignal).
- Public: receive alerts like "Your area is at high risk for dengue in the next 7 days."
- Health officials: get predictive alerts for previous resource planning.

5. Multilingual Awareness Module

- Sinhala, Tamil, and English content.
- Scenario-specific tips (e.g., "Clear coconut shells after rain" in Sinhala situation).
- Displayed in-app + in push notifications.

6. Chatbot Assistant

- Built-in chatbot in the mobile app (Dialogflow/Rasa).
- Answers FAQs: symptoms, outbreak status, precautionary measures.
- Makes it more accessible to less tech-savvy users.

4. Gantt Chart & Work Breakdown Structure

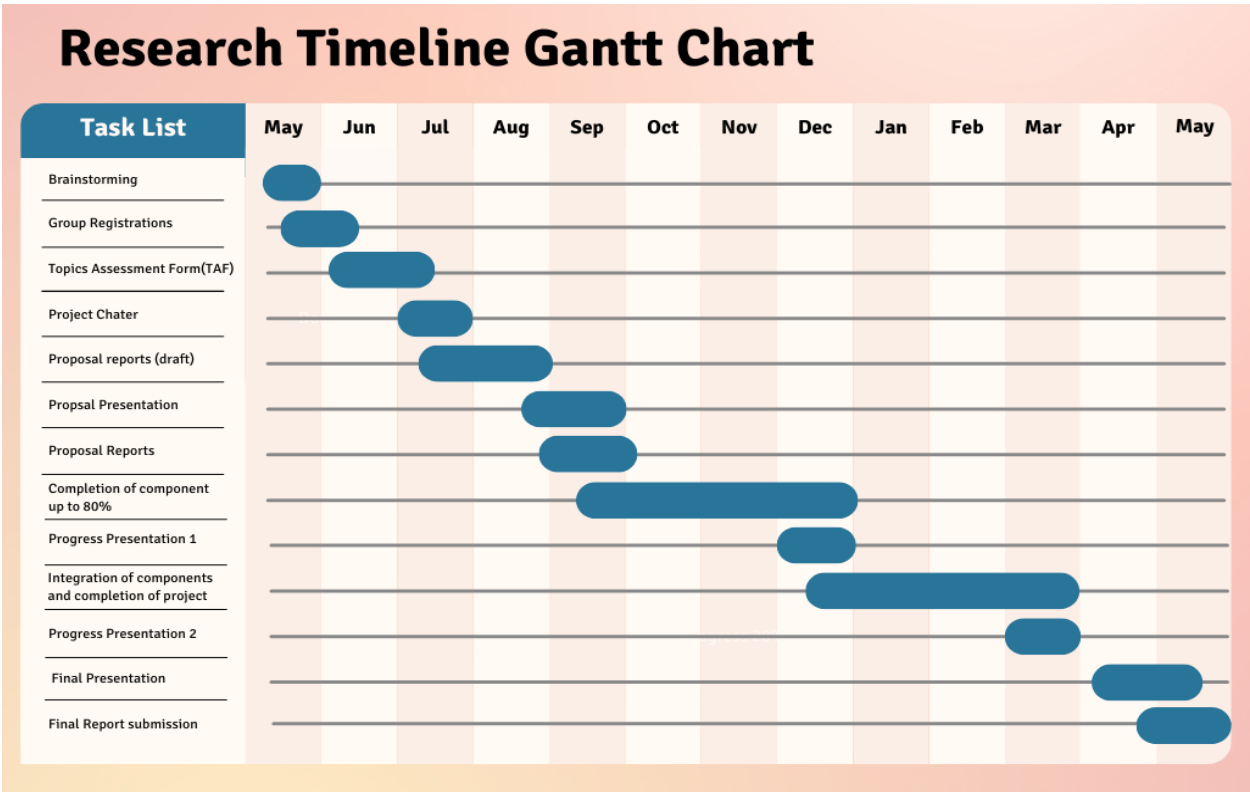


Figure 3 - Gantt Chart

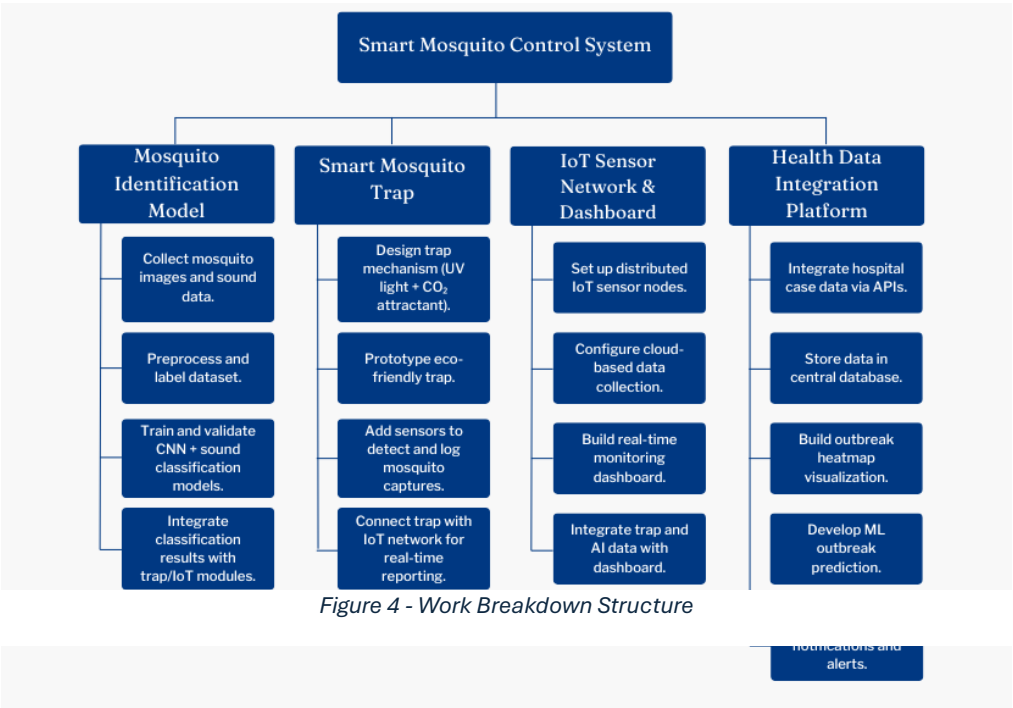


Figure 4 - Work Breakdown Structure

5. Project Requirements

5.1 Functional Requirements

Functional requirements specify the necessary attributes and functionalities that the system must perform. They define what the system must do in order to meet its function and give the desired output. In this project, the functional requirements specify how the health data integration platform will gather, process, and present hospital data and how it will communicate with the users.

1. Hospital Data Integration

The system needs to establish a secure channel to retrieve real-time dengue and chikungunya case information from hospitals. This can be either directly through hospital information system APIs or through manual entry by the respective staff. The received data should be stored in a central database so that it is immediately made available for visualization and analysis.

2. Outbreak Heatmap Visualization

The system should provide an interactive map that has real-time updates to accommodate outbreak cases. Using the GIS technology, the heatmap should reflect areas of different risk (low, medium, high) based on case density. This will provide an easy way for both members of the public and health officials to identify hotspots.

3. Machine Learning Prediction

Its machine learning module has to sift through historical case data and weather data such as rain, temperature, and humidity in order to forecast possible future outbreaks. The system should then highlight forecasted high-risk hotspots in the heatmap, giving users a future-oriented view of possible health threats.

4. Notification and Alert System

The system must produce timely alerts to various user groups. The health officers must be provided with detailed alerts of the estimated outbreak

location for early decision-making and resource planning. The general public must be provided with basic alerts of the potential for an outbreak in their local area, to alert them to take precautionary measures

5. Multilingual Awareness Module

The system should provide culturally suitable health guidance along with prevention interventions in Sinhala, Tamil, and English. All Sri Lankan communities would be in a position to access the information regardless of the preferred language. The messages should also be personalized based on local customs, e.g., draining coconut shells or water tanks to remove breeding sites.

6. Chatbot Assistant

A chatbot supported by AI should be included in the mobile app. The chatbot will allow the users to ask questions regarding dengue and chikungunya outbreaks, their symptoms, and how to prevent them. It should be multilingual (English, Sinhala, and Tamil) to allow wider reach and usage, especially for non-technical users.

5.2 Nonfunctional Requirements

Non-functional requirements specify the quality attributes of the system and not the system's functionality. Non-functional requirements establish how the system should behave, e.g., speed, reliability, security, scalability, and usability. In this project, non-functional requirements ensure that the health data integration platform is secure, efficient, and easy to use for the public and health authorities.

1. Performance

The system needs to update and process outbreak data from neighboring hospitals in near real-time with latencies of no more than several seconds. Heatmap and other visualization components need to load in 2–3 seconds or less so that user interaction can be smooth even during heavy load.

2. Reliability

The site should be highly available with the minimum availability of 99%. The messages and notifications should always arrive at the destination intact, serving as a trusted means of communication for the users and the system.

3. Scalability

Because the system will expand to accommodate larger numbers of hospitals and users, it will need to be able to process increasing amounts of data and concurrent user requests. Design will need to be able to scale from pilot deployment across a group of multiple hospitals to scale-out to deployment across an entire country with minimal re-engineering.

4. Security

Hospital information is confidential and should be protected. All data exchange must use encrypted communications (HTTPS/SSL). Storage of data in the central database must be encrypted, and user access must be managed with role-based access controls (e.g., health officers see detailed information, but the public sees summarized outbreak information).

5. Usability

The user interface must be designed to be simple, intuitive, and user-friendly. The information regarding outbreaks and prevention tips must be readily accessible to public users while more detailed dashboards and reports must be provided for health officers. Sinhala, Tamil, and English must also be supported on the platform so that maximum accessibility can be achieved.

6. Portability

The system should be accessible on every platform and device. The responsive web app should be supported by all browsers in use, and the mobile app should be supported by Android and iOS users.

7. Maintainability

The platform should be structured with modular architecture in a way that changing or enhancing the components does not affect the entire system. As an example, the machine learning prediction model should be updateable whenever better models are created, and chatbot responses should be easily updated as health regulations change.

6. Reference list

- [1] World Health Organization, "Dengue and severe dengue," WHO Fact Sheets, 2024. [Online]. Available: <https://www.who.int/news-room/fact-sheets/detail/dengue-and-severe-dengue>
- [2] Ministry of Health, Sri Lanka. "Dengue Update," Epidemiology Unit, 2023.
- [3] National Environment Agency (NEA), Singapore. "Dengue Cases and Clusters," 2023.
- [4] Palmer, J.R.B., et al. "Citizen Science for Mosquito Control: Mosquito Alert." Nature Communications, 2017.
- [5] Guo, P., et al. "Developing a Dengue Forecast Model Using Machine Learning: A Case Study in China." PLoS Neglected Tropical Diseases, 2017.
- [6] Xu, J., et al. "Time-Series Analysis and Forecast of Dengue in Southeast Asia Using Climate Data and LSTM Models." Scientific Reports, 2020.

7. Appendices

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